STRATEGIC DOSSIER VOLUME 3

SURGCLEAR PLATFORM MARKET GAP & COMPETITIVE MOAT

Why There Is Nothing Like SurgClear in South Africa Today: Industry Inefficiencies, Legacy Incumbent Failure, and the Unfair Aggregator Advantage.

Solving the R15 Billion South African Surgical Billing Black Hole & Eliminating Theatre Billing Shock

DOCUMENT TYPE
Competitive Intelligence & Market Strategy

TOTAL ADDRESSABLE MARKET (TAM)
R 1.44 Billion ZAR Annual Transaction Software

MARKET OPPORTUNITY
1.2M Elective Private Surgeries / Year

CURRENT STATE OF INDUSTRY
Fragmented, Paper-Based, Adversarial Billing

PRIMARY INCUMBENTS
Healthbridge, MediSwitch (Altron), Discovery App

COMPETITIVE ADVANTAGE
Multi-Stakeholder Clearance & Auto Gap Filing

1. The South African Healthcare Dilemma: Where the Gap Is

South Africa's private healthcare system is internationally renowned for clinical excellence, yet operationally crippled by **financial opacity and administrative friction**.

> **The Anatomy of "Theatre Billing Shock":** When a patient is booked for elective surgery (e.g. knee arthroscopy, gallbladder removal, spinal fusion), they receive a generic quote. Four weeks after discharge, the patient receives unexpected invoices totaling R15,000 to R45,000 for anaesthetic surcharges, surgeon tariffs billed at 300% of scheme rates, and out-of-network hospital penalties.

Why the Market Has Remained Broken: The 4 Industry Conflicts

- **1. The Doctor-Scheme Cold War:** Medical schemes pegged their reimbursement tariffs to outdated National Health Reference Price Lists (RPL), effectively cutting doctors' real incomes by 30% over a decade. Specialists responded by raising private billed rates to 200%–350%. Schemes refuse to negotiate; doctors refuse to discount. The patient is caught in the crossfire.
- **2. The Hospital DSP Trap:** To contain costs, medical schemes established Designated Service Provider (DSP) hospital networks. If a patient is admitted to an acute hospital that is non-network for their plan tier, schemes impose a punitive **R8,000 to R15,000 co-payment deductible**. Hospitals often fail to warn patients in advance, leading to catastrophic debt collection.
- **3. The Gap Cover Paperwork Barrier:** While over 2.5 million South Africans have purchased Gap Cover (Sirago, Turnberry, Zestlife) to insure against specialist shortfalls, claiming is entirely manual. Patients must gather three separate documents: the hospital bill, the doctor's itemized invoices, and the medical scheme's electronic Remittance Advice (RA). Over **35% of eligible Gap Cover claims are never filed** due to administrative exhaustion.
- **4. Hospital Admissions Chaos:** Hospital intake desks between 06:00 and 08:00 AM are plagued by queues, paper clipboards, physical indemnity signing, and last-minute dietary confusion (Halal, Kosher, Diabetic).

2. Why There is Nothing Like SurgClear: Incumbent Comparison

Existing healthcare software platforms solve only narrow, single-sided problems. None unify the specialist trigger, the medical aid switch, the Gap insurer, the hospital desk, and the patient:

Capability / Feature	Legacy EDI Switches (Healthbridge / MediSwitch)	Medical Scheme Portals (Discovery / Medscheme)	Hospital Web Portals (Netcare Online Intake)	SurgClear Platform (Our Solution)
Core Function	Doctor-to-scheme claim switching for consultation billing	Member self-service & scheme benefit tracking	Basic biographical data capture for admissions	**Unified multi-stakeholder surgical financial clearance**
Pre-Op Shortfall Calculation	❌ None (post-facto only)	❌ Only shows scheme's payout (ignores doctor's 300% rate)	❌ None (hospital ignores doctor bills)	✅ **Real-time 100%-300% shortfall & net patient balance**
Gap Cover Integration	❌ Zero gap cover connectivity	❌ Actively hostile (Gap is considered competitor product)	❌ Zero gap cover connectivity	✅ **Evaluates policy against R223k OAL; auto-files claim post-op**
Hospital Network DSP Check	❌ None	⚠️ Shows list, but does not calculate exact penalty in ZAR	⚠️ Facility-specific only	✅ **Instant DSP network validation + penalty calculation**
Patient Mobile Pass & QR	❌ No patient-facing interface	⚠️ Scheme digital card (not an admission boarding pass)	⚠️ Web form confirmation number	✅ **4-step mobile PWA, meals, consent & high-contrast QR pass**
Dietary & Ward Hospitality	❌ None	❌ None	⚠️ Generic checkbox (often ignored by kitchen)	✅ **SANHA Halal / Beth Din Kosher / Soft meal selection sync**
Medical Shortfall Financing	❌ None	❌ None	❌ None	✅ **Integrated 6-Month Medical Financing calculator**

3. The Unfair Competitive Moat (Why SurgClear Wins)

- **The Dual-Wedge GTM Moat:** By offering specialist surgeons a friction-free 30-second booking cockpit, SurgClear captures surgical episodes at the point of origin. Once specialists mandate SurgClear, hospital admission desks and Gap underwriters are compelled to integrate.
- **Data Network Effects:** With each processed surgical episode, SurgClear refines its algorithmic model of hospital DSP networks, scheme tariff schedules, and specialist billing rates, creating a proprietary actuarial database that competitors cannot replicate.
- **Neutral Switzerland Positioning:** Medical schemes cannot build SurgClear because specialists do not trust them; hospital groups cannot build it because specialists practice across competing networks. SurgClear is the independent, trusted aggregator.